

COOK COUNTY CHILDREN'S SPECIALTY CENTER

Division of Pediatric Otolaryngology — Head and Neck Surgery

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ENT CONSULTATION NOTE

Patient: Marcus D. Whitfield MRN: PED-2025-04471

DOB: 03/03/2016 Age: 9 years, 10 months Sex: Male

Date of Visit: 01/30/2026 Visit Type: Consultation

Referring Provider: ED referral (01/22/2026 visit)

Attending: Michael Torres, MD — Pediatric Otolaryngology

REASON FOR CONSULTATION

Evaluation of tonsillar hypertrophy (3+) identified during ED visit on 01/22/2026. Caregiver reports nightly snoring and witnessed apneic episodes during sleep.

HISTORY OF PRESENT ILLNESS

Marcus is a 9-year-old male with ASD Level 2, ADHD, PTSD, FASD, severe persistent asthma (3 intubations), and suspected Crohn's disease referred for ENT evaluation after tonsillar hypertrophy was noted incidentally during an ED visit for asthma exacerbation on 01/22/2026. His foster parent reports that he snores loudly every night, with witnessed pauses in breathing during sleep followed by gasping. She describes restless sleep with frequent awakenings and position changes. Marcus appears fatigued during the day and his teacher reports that he falls asleep in class almost daily. He has never had a formal sleep study. A polysomnography referral was placed by his prior PCP (Dr. Sharma) on 01/15/2025 but was never scheduled and has since expired.

PAST MEDICAL HISTORY

- ASD Level 2
- ADHD, Combined Type
- PTSD
- FASD
- Severe persistent asthma — 3 lifetime intubations (most recent 06/2024)
- Suspected Crohn's disease (infliximab on clinical hold)
- Pica with elevated blood lead (18 mcg/dL at 01/22/2026)
- NF1 (genetic evaluation ongoing)
- Iron deficiency anemia

CURRENT MEDICATIONS

- Risperidone 1.5 mg QHS
- Methylphenidate ER 27 mg QAM
- Clonidine 0.1 mg QHS
- Budesonide/Formoterol 80/4.5 mcg BID
- Albuterol MDI PRN
- Montelukast 5 mg QHS
- Ferrous sulfate 325 mg daily
- Melatonin 3 mg QHS (OTC)

EXAMINATION

HEENT: Tonsils 3+ bilaterally, noninjected, no exudate. Adenoid facies not prominent. Nasal passages patent bilaterally, no septal deviation, no polyps. Tympanic membranes clear bilaterally, no effusion, no retraction. Mallampati class III. Oral cavity: mild crowding of dentition, no oral lesions. Neck: Supple, no cervical lymphadenopathy, no thyromegaly.

General: Thin male, quiet, limited verbal interaction. Intermittent hand-flapping noted. Dark circles under eyes. Breathing comfortably through mouth at rest.

ASSESSMENT

Tonsillar hypertrophy (3+) with a clinical history highly suspicious for obstructive sleep apnea in a child with multiple risk factors including ASD, FASD, and neurodevelopmental disability. Adenotonsillectomy is the first-line surgical treatment for pediatric OSA in the setting of adenotonsillar hypertrophy, and the evidence for benefit is particularly strong in children with neurodevelopmental comorbidities who are at elevated baseline risk for sleep-disordered breathing.

PLAN

- Polysomnography (PSG) must be completed prior to surgical planning. If PSG confirms moderate-to-severe OSA (AHI > 5), adenotonsillectomy is recommended. A new PSG referral is required since the prior referral (01/15/2025) has expired without being scheduled.
- Surgical timing considerations: Marcus is currently on infliximab (on clinical hold) for suspected Crohn's disease. If immunosuppressive therapy is active at the time of surgery, perioperative infection risk must be assessed and coordinated with GI. Any outstanding infectious workup should be completed prior to elective surgery.
- PCP to coordinate new PSG referral to Cook County Sleep Medicine Center.
- Follow-up in this clinic after PSG results are available for surgical planning discussion.

Michael Torres, MD

Pediatric Otolaryngology

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Electronically signed: 01/30/2026 at 11:15 CST